

This handout is for educational reference only. It is not medical advice. Always follow your prescription label, your pharmacy-specific instructions, and your KORB provider's guidance. If you have questions, contact KORB using the details at the end of this handout.

What Hormone Therapy is

Hormone therapy replaces hormones your body has stopped making in the amounts it used to. Around menopause, estrogen and progesterone fall, and testosterone falls too. That decline is what causes hot flashes, night sweats, disturbed sleep, vaginal dryness, low mood and the difficulty concentrating many women describe as brain fog.

The hormones KORB prescribes are bioidentical, meaning they are structurally the same as the ones your body makes. You may be prescribed one, two or three of them, and they are counted that way when your programme is priced.

Most of them are absorbed through the skin rather than swallowed. That matters: going through the skin avoids the first pass through your liver, which is why a patch or cream carries a lower risk of blood clots than an oral estrogen does.

What it may support

Hormone therapy is prescribed for symptoms that are affecting your quality of life, most commonly:

- Hot flashes and night sweats
- Sleep that is broken or unrefreshing
- Vaginal dryness, or discomfort with intercourse
- Low mood, irritability, or mood that swings more than it used to
- Difficulty concentrating - brain fog
- Low libido, low energy, loss of muscle tone

How to use it

Your prescription label, or the instructions given to you by your KORB clinical provider, is the authoritative source for your dose and directions. Some pharmacies label vials "as directed by provider" rather than printing exact instructions. What follows is a general reference. Do not adjust anything without speaking to your provider first.

How to inject

Patch, cream or capsule, depending on which hormones you are prescribed

When to inject	Creams and patches in the morning unless told otherwise. Progesterone capsules at night.
Schedule	As stated on your prescription label

The hormones you may be taking

Hormone	What it is for	How you take it
Estradiol	The main estrogen. Treats hot flashes, night sweats, sleep and vaginal dryness, and supports bone.	A patch changed once a week, or a cream you apply daily.
Progesterone	Protects the lining of the uterus, and helps sleep.	A capsule taken at night.
Testosterone	Supports libido, energy, mood and muscle. Women make it too, and it falls at menopause.	A cream you apply daily. Available in Texas and California only.

Why progesterone matters if you still have a uterus

If you still have a uterus, you will always be prescribed progesterone alongside estrogen. Estrogen on its own thickens the lining of the uterus, and over time that can lead to cancer. Progesterone prevents it. This is not optional and it is not a preference - if you have been prescribed estrogen without progesterone and you have not had a hysterectomy, contact us before you start.

How to apply a cream

Step	What to do
1	Wash your hands.
2	Apply to thin, hairless skin - the inner thigh, the inner forearm, or the back of the upper arm.
3	Rub it in and let it dry completely before you dress or touch anyone.
4	Wash your hands again, thoroughly.
5	Rotate where you apply it. Using the same patch of skin every day reduces how well it absorbs.

One click is one measured dose. Do not guess at it and do not double up if you miss one.

How to use a patch

Storage and handling

What to do	Detail
Store at room temperature	68°F – 77°F (20°C – 25°C)
Do NOT refrigerate	None of these need cold storage
Keep the cap on the pump	And keep creams upright
Out of reach	Of children, and of anyone else in the house

Your prescriptions are written as a 12-week supply. If you have medication left at the end of 12 weeks, that is expected on the creams - the bottle holds a little more than 12 weeks of doses. Start the new bottle when it arrives rather than finishing the old one.

Traveling with your medication

Hormone therapy travels easily. Keep everything in your carry-on rather than a checked bag, leave it in its original labelled container, and do NOT refrigerate it at any point - these are room-temperature medications and cold is not better for them. If you are crossing time zones, keep your patch change day and your nightly capsule on your home schedule until you are settled.

Side effects and what to watch for

What you may notice

What you may notice	What to do
Breast tenderness or swelling	Common in the first few weeks. Tell us if it does not settle - it often responds to a small change in dose.
Headache	Usually early and short-lived. Tell us if it is new for you or persistent.
Bloating or fluid retention	Common early. Reducing salt and keeping hydrated helps.
Mood changes	Tell us. Mood is one of the things dose adjustment tends to fix.
Nausea	More common with capsules. Taking it with a little food at night helps.
Spotting, or a change in your bleeding	Expected in the first months, especially if you are cycling progesterone. Tell us at your visit, and sooner if it is heavy.

What you may notice	What to do
Acne, or more hair growth on the face or body	Usually testosterone. Tell us - the dose can come down.

Tell your KORB provider at your next visit

What you may notice	What to do
Bleeding that is new, heavy, or after you had stopped altogether	Always tell us. It needs looking at rather than waiting for your visit.
A lump in the breast, or breast pain that does not settle	Tell us promptly. We will arrange the right assessment.
Mood that is getting worse rather than better	Tell us. Do not wait out a low mood on the assumption it will pass.
Headaches that are new or unusual for you	Tell us before your next dose.
A change in your voice, or hair growth you are unhappy with	Tell us early. Voice changes do not reverse, so we would rather hear about it at the first sign.
No improvement at all after 12 weeks	Tell us at your visit. It usually means the dose or the route needs changing, not that hormone therapy will not work for you.

Call 911 or go to an emergency room if you have any of these:

Lab monitoring

KORB uses at-home saliva collection rather than a blood draw for hormone levels.

- A kit is sent to you. You collect at home and post it back.
- Saliva is used deliberately. It measures the fraction of the hormone that is actually reaching your tissues, which is what matters when the hormone is going through your skin. A blood level barely moves after a cream, so it would tell your provider very little.
- The complete panel adds a small finger-prick blood spot for thyroid, because thyroid problems cause many of the same symptoms and are worth ruling out.

Safety reminders

- Tell your provider before you start if you have had breast cancer, endometrial cancer, or any cancer that was sensitive to estrogen.
- Tell your provider if you have had a blood clot or a clotting disorder, or if a close relative has.

- Tell your provider if you have had a stroke or a heart attack, or if your blood pressure is not controlled.
- Tell your provider if you have liver disease.
- Tell your provider about any vaginal bleeding that has not been explained.
- Tell us if you have a PEANUT ALLERGY. The commercial progesterone capsule is made with peanut oil. A compounded capsule is not, and we will prescribe that one instead - so this does not stop you having progesterone.
- If you still have a uterus, never take estrogen without progesterone.
- Do not change your own dose, and do not use anyone else's hormones.
- Use a new insulin needle and syringe for every injection. Do not reuse.
- Never mix peptides. Use a separate needle and syringe for each peptide and each injection; do not combine them into one syringe.
- Follow your state or local rules for sharps disposal. Use an FDA-cleared sharps container, and do not put used needles or syringes in household trash unless your local guidance specifically allows it.

When to contact KORB

Contact KORB Operations	Patient Portal — message your provider	Seek emergency care immediately
• Questions about timing, storage or administration • Scheduling, billing and shipping Email: info@korbhealth.com Phone: +1 (888) 959-7299 Hours: Mon–Fri, 9 AM–6 PM CT	• Injection-site irritation or redness that does not resolve • Mild but persistent side effects • Questions about whether to continue therapy	• Trouble breathing, or swelling of the face, lips, tongue or throat • Severe rash or widespread hives • Chest pain or fainting • Severe or rapidly worsening symptoms • Confusion or severe weakness • Any symptom that feels urgent or unsafe

The Patient Portal is the only HIPAA-compliant way to message your provider directly. Providers check portal messages once per day on their clinic days. For anything that does not need your provider's direct medical judgment, contact KORB Operations by phone or email instead.

Phone and email are not appropriate for emergencies. When in doubt, go to urgent care or the emergency room.

Key reminders

- If you have a uterus, never take estrogen without progesterone.
- Let creams dry fully before contact with anyone else, and wash your hands.
- Progesterone is taken at night. It helps you sleep.
- Your prescriptions run 12 weeks, and your follow-up is at 12 weeks.

- Do not change your own dose. Tell us what you are feeling and we will change it with you.
- Nothing here needs refrigerating.